UNIT CODE: DHRIT 2111

UNIT TITLE: HEALTH RECORDS MANAGEMENT 1

Introduction

The course is designed to equip health records and information students with knowledge, skills and attitude to enable them to organize and manage Health records & Information services.

Objectives

By the end of the course the student should be able to;

1. Give historical background of health records discipline and its organization
2. Explain the values and uses of Health records
3. Display the qualities of a Health records & information officer & his accountability
4. Identify & manage different types of health records and other sources
5. Describe and display ability to manage health records functions & interdepartmental relationship.
6. Organize and manage the reception, registration and initiation of patients/clients health records
7. Organize and manage health records indices
8. Organize and manage patients /clients schedule and follow ups
9. Organize and manage clinic preparation procedures
10. Organize and manage the admission and discharges of patients /clients
11. Organize and manage the storage, retrieval control & maintenance of health

records

12. Demonstrate the ability to analyze & interpret, safeguard legal aspects of health records, various acts related to them
13. Acquire and maintenance of health records equipment
14. Manage special Health records
15. Participate in the organization and management of other records
16. Establish a health records department in a health care facility
17. Design medical forms and manage document reproduction- DHIS
18. Ensure quality control & assurance in Health records & health care services
19. Organize & Manage medical secretarial services.

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Historical background of health records system and its organization

Learning Objectives

I. Define health records

II. Give historical background of health records

Definition of Health records: Any written document about a patient in a professional relationship with a doctor.

-The history of health records runs parallel with the history of medicine. Records are necessary for practice of medicine as medications are for effective treatment, and they seem to have been made from earliest antiquity

- The first real physician of health records in Egypt is Imhotep. He lived in the pyramid age (about 3000-2500 BC) and was grand vizier, chief architect and royal medical adviser to pharaoh of the 29th century BC. Imhotep has been credited with the authorship of the Edwin Smith Papyrus. This Papyrus is one of the most valuable ancient medical documents that has come down to us and appears to have been copied about 1600BC from an earlier original. This papyrus is a roll over 15 feet by about 3 inches wide and is made up of 12 sheets of the usual size. It is written on both sides and consists of 48 cases of clinical surgery.

-In Greece Hippocrates known as the "Father of Medicine" was born about 460 BC. He was the first to cast out superstition and to practice medicine on scientific principles. He was the author of the Hippocratic Oath, which is pledge by physicians and which states in part "Whatsoever in my practice or not in my practice, I shall see or hear amid the lives of men which ought not to be noised abroad as to this I shall keep silence, holding such things unfitted to be spoken". Thus originated the privacy of all information given to physicians by patients, eventually the Health Records per see was also considered as privileged communication Hippocrates kept detailed case reports of his patients.

- In the 18th Century, Benjamin Franklin was one of the first leaders in the movement to

establish the first incorporated hospital within the United States. This institution was known as Pennsylvania in 1751. Franklin served as secretary of hospital, and many of its earliest records are in his handwriting. In 19th Century in 1821 the famous Massachusetts General Boston Opened it has the distinction of having a complete file of clinical records, with all cases catalogued dating from the day it opened.

- In the 20th Century, it was not until the beginning of this era that medical records received serious consideration by other types of hospitals and especially by hospitals and medical associations. In 1902 the American Hospital Association discussed health records for the first time at a convention. In 1948 Health Records Association was started in Britain. In 1967 the Department of Health Records was started in Kenya at Kenyatta National Hospital- Nairobi.

In 1978 the Health Records and Information Technician Program was started at the Medical Training College, Nairobi. In 1990, the Health records and information officers program was started at the medical training college, Nairobi.

UNIT HEALTH RECORDS SYSTEM

The health records systems where one patient is given one number for the rest of his stay or attendance in one hospital.

Learning Objectives

I. Define the Unit Systems

II. Describe the health unit system

III. Initiate the unit record

IV. Prevent the duplication of the unit number

V. Describe the contents of the unit health records

Definition of the unit system -The unit system is a health records system in which all health records notes relating to one patient are contained in one case folder- The patient is the unit.

Initiation of the unit system

This is whereby it is necessary to assign a unit number to every patient attending for the first time. A six digit range of number is normally used ranging from 1 to 999,999.When the old patient attend or is admitted at the first time his old notes are filed at the back of his new unit folder under the new unit number. Only the old records of patient who present themselves after the appointment days are brought in this way, the other remaining in the old file under their original identification number. A tracer card should be replaced where old record was indicating the new unit number

Prevention of duplication

Duplication in the use of unit number is prevented through the following;-

- Either by not giving the same number to two patients
- Not giving the same patient more than one number. In addition the patient should be asked whether he has attended the hospital previously during registration. Whether the patients says yes or no across checking can be done through the patient master index to ascertain this.
- Records are contained in one case folder
- Has six digits

The unit number register

The importance of unit system is appreciated if the unit number register is maintained.

The unit number register have the following information:

a) Full names of the patient

b) Address

c) Date of birth

d) Registration

e) Specialty to which they have been referred

Contents of the health records

A. The folder

When selecting a folder the following should be considered:

i. Strength of manila

ii. Method of fastening in documents

iii. Clarity of numbering

iv. Cost of the folder

The folder should be made of tough manila with a gusset and single or double fold inside through which are threaded metal or plastic prongs with which the loose sheets may be fastened into the folder

B. The front sheet or identification sheet

This is the area where the patient social details are transcribed from the preregistration form. Space should be provided to record several changes or address and change of a doctor. A section for listing consultants whose clinics the patient has attended or wards. The facts should be readily visible to the doctor who is in charge of the patient. The information should be complete and accurate for easy in follow up.

C. Clinical history sheet

i) Consultants/Clinical history sheet

Each consultant has his individual history on which he only will write. A patient attending three different clinics will have three separate history sheet though they will be filled next to each other. The consultants can also use continuous history sheets. The advantage is it's easy to identify by using different coloured stamp/clinic stamp.

ii) Continuous history sheet

This is the sheet whereby every doctor writes as he sees the patient

D. Continuation sheet

Continuation sheet is similar to the history sheet but without any special printed identification of

Consultant or clinician which enables them to be used by all consultants

E. Prescription chart

This makes formal provision for the doctor to prescribe drugs for inpatient. The form must be taken back to the case folder from the pharmacy so that the doctor treating the patient can see the previous medication that has been prescribed

F. Surgical operation sheet

This sheet contains sections in which the names of all the surgeons and anaesthetics taking part in the operation are to be recorded and special remarks about drainage, blood loss, special recovery treatment etc.

G. Anaesthetic record

This form contains considerable amount of detail filled in by the anaesthetist during the operation and immediate post-operative period

H. Temperature, Pulse and Respiration (TPR) chart

There are usual two versions of this chart

i) B.D-twice daily chart for the recording of the routine observations urine and stool details are usually recorded at the foot of the TPR chart. A separate chart is often used for four hourly recording for more seriously ill patients. This is frequently printed on coloured paper or in coloured ink for it to be noticed

I. Report mounting sheets

This is where forms which are filled in the folder are usually mounted on a report mount sheet. They vary in designs and method of gumming. They are gummed on the edge and stacked on the mount sheet. The forms are filed one above the other, but approximately one quarter inch or one third inch of the lower edge left visible so that the date and type of report can immediately be seen. In this way, a dozen or more reports will be filed on one mount sheet

J . Consent form

This is written consent which is obtained for all operative procedures, sterilization procedures, post mortem examination. It is obtained from the patient's relative or parent (if the child is under
years) it is mounted on the mount sheet

K. Report forms

These are investigation report forms e.g. laboratory, radiology results etc. They are fastened into patient notes

L. Clinical photographs

They are taken usually when the patient has an unusually interesting condition e.g. in plastic surgery, patient pass through several stages of treatment, the photograph is taken in intervals to record the progress. They are filed in the case folder

M. In – patient summary

This is the record which is done after the patient has been discharged from the hospital. It is done by the registrar or consultant to dictate a summary of the salient points of the patients progress while an inpatient, operations done or other treatment. A copy of this summary is often used as the hospital's full report to the general practitioner. This forms the document on which diagnostic coding is carried out

N. Correspondence

Copies or originals of the communications from general practitioner to hospital doctors and from hospital to general to general practitioner must be returned including copies of discharge note

CHAPTER THREE

DIFFERENT TYPES OF HEALTH RECORDS- THEIR VALUES, USES & RESPONSIBILITIES OF A HEALTH RECORDS & INFORMATION OFFICER

Learning Objectives

1. Identify different types of health records
2. Explain the uses and values of health records
3. Explain the qualities and responsibility of health records & information officer

Types of health records

i) Case record (patient record)

These are records initiated for the patients who get admitted into the wards or who continuously
attend the various consultants clinic. The information contained in the case records can be expressed verbally, graphically, diagrammatically or in tabular form. This depends on the person who is taking the information

ii) Outpatient record

This includes all the cards that are used in the outpatient department e.g. casualty card, acute natal cards or any other card may be used in the outpatient department

iii) Diagnostic records

This included notes on radiography, pathology, electrocardiography and any other investigations that are usually initiated by report forms. The forms is 6”x4”in size and are available in all the consultation rooms both in the outpatient and inpatient department. They are in different colours for quick identification

VALUES AND USES OF HEALTH RECORDS

Health records have various uses:

1. Treatment

Health record is the value in the present and future treatment of the patient. The individual record is a reminder to the physician surgeon, clinician etc. It contains what has been observed by the others. It provides or should provide the whole previous hospital record of the patient, so that every fact that may be important is permanently available for reference at any time

2. Planning

Health statistics and information from the health record will definitely be very useful in the planning of health services. The statistics should be very accurate and disseminated promptly to the user

3. Research

Accurate recording observation in the health records will lead to accurate information required for research. The methods of filing and retrieving the records should be systematic and simple

4. Teaching

Health records can be used as an educational tool or instrument. When the quality of health records is high, the task of the students is simplified and conversely, when the quality is poor the student's task is made more difficult and his progress thereby impended

5. Administration

Complete health record means increased and efficient service to the public avoiding vexation, litigation and fair settlement of claims and capacity enquiries about the hospital work.

For the record to be useful for the above purpose it should be designed for the purposes for which it is to be used. The record should meet both the clinicians and the record personnel requirement. The record should be complete i.e. analyzed quantitatively by the records officer and qualitatively by the clinician

QUALITIES OF A HEALTH RECORDS & INFORAMTION OFFICER

Integrity-Honest, morally upright

Adaptability - Should be flexible to the circumstances in an environment

Discretion - Ability to use his own knowledge to make decisions

Politeness - Humility, humble

Calmness - A state of not being emotional in an emergency situation

Neat appearance - Smart and acceptable appearance in his work

Consistent efficiency - Effective and exceptional performance in his work

Personal sense of vacation and service other health workers and hence the patient

FUNCTIONS OF THE HEALTH RECORDS AND INFORMATION DEPARTMENT

By the end of the session, participants should be able to:-

i. Identify the main function of Health Records and Information Department

ii. Relationship between Health Records Department and other Departments

1. Reception

This is the area where patients/clients are welcomed and sorted and directed elsewhere for treatment or other services

2. Registration

This is the recording of identification and social details of the patient/client i.e. name, address, sex, residence and other identification information

3. Admission

This is the procedure carried out when the patient is required to be confirmed to a hospital ward/bed for further treatment or service

4. Discharge

This is the procedure that is carried out when the patient has undergone treatment or consumed

services and has been allowed to return to their home/residence

5. Appointment

This is the procedure that allows for the patient/client who has been ordered by the attending doctor/clinician to return to the hospital at a future date for treatment

6. Filing

This is a systematic way of keeping individual health care document and aggregate documents in

appropriate equipment and using appropriate methods

7. Tracing

This is the control of the movement of patient/client records or documents

8. Clinic preparation

This is the getting ready all patient/clients documents at least two days before the client/patient

returns to hospital for treatment or other service

9. Follow up

This is a mechanism put in place so that patients/client diagnosed with certain diseases/conditions usually long term, or infectious and who have been discharged are treated away from the hospital, either at their homes or nearby hospitals to ensure the disease/condition is monitored

10. Coding and indexing

This is the allocation of symbols/numbers that are predetermined for each condition to the disease that the patient/client is suffering from: This code is then recorded together with all similar disease hence indexing. Collection tabulation, analysis, interpretation and dissemination of health care data. Data on individual patients and on segment of/or the who, population is gathered processed, made meaningful and given to those who need the information

11. Maintenance of health record equipment

All machines: tools, technology and so forth supports the management of information is taken care of by the Health Records and information Department. Maintenance of

information confidentiality, safe custody of all patients/clients Information, captured in all media, to prevent unauthorized disclosure, is one of the Health Records and information Department function

12. Manage special health records

Tuberculosis, maternity, genitor-urinary, HIV/AIDS, assault etc. information is usually given special attention

13. Design medical forms and computer views (E.g. DHIS)

The Health Records and Information department is expected to ensure capturing of information is neat, accurate, and consistent. This is achieved by capturing the data uniformly in standard forms and in consultation with the users.

14. Ensuring quality assurance

The Health Records Information Department ensures that the highest standards of information capture, use, maintenance/storage and disposal are established and maintained

15. Coordination with other departments

As may be noted during this HR&I Professionals training and working. The HR&I is one of a larger team of people who render services to the patients/clients. Because all other departments entrust the HR&I department with this keeping of information they captured. Coordination is important; no one department can serve patient or its own

RELATIONSHIP BETWEEN A HEALTH RECORDS DEPARTMENT AND OTHER DEPARTMENT IN A HEALTH FACILITY

The medical records officer in a health facility is expected to co-ordinated the day

administrative functions with other departments in a hospital. Some of the departments in a health facility are listed below:-

1. Laboratory
2. X- Ray
3. Radiography
4. Orthopedic
5. Dental
6. Physiotherapy
7. Occupational Therapy
8. Department of Nursing
9. Department of Oral Health
10. Supplies
11. Accounts
12. Transport
13. Maintenance
14. Nutrition
15. Pharmacy

All this co-ordination is done to enhance the treatment of the patient. Therefore the stands in the middle and all these activities revolve around the patient. No one department is more important than the other, they should work together towards the achievement of this common goal.

EFFECTIVE COMMUNICATION

This chapter is an important one since it is a management function that the health records officers needs in his/her day to day work. It covers communication process,

skills and barriers to effective communication.

Learning Objectives

i. Define communication
ii. Explain why we communicate
iii. Describe the communication process
iv. Describe the methods and media of communication
v. Explain barriers to effective communication
vi. Explain how to overcome these barriers

Define

Communication- Is the transmission and reception of messages or
Any means by which a thought may be transferred

Objectives of communication

a) To get others to think or act the way we want them to think or act
b) To inform others
c) To ask and answer question
d) To listen to others (Listening is one of the most important communication skills)

Communication Process

We spend most of our lives communicating in to someone or another even when we are alone. We are picking up communication through our senses. The average manager or supervisor spends up to 80% of his time communicating during the working day.

METHODS AND MEDIA OF COMMUNICATION

There are at least nine different ways in which we can communicate or receive a message. They are;-Speech, Sight, Sound, Action, Mannerisms, Taste, Smell, Touch, Silence.

Speech

Is an obvious method of communication usually is the one we think of first.

Sight

Is the most effective of all methods of communication because the mind thinks in pictures. The average person is able to remember a mental picture of up to 80% of what he has seen. Usually he can only remember or grasp 20% of what he has heard. This is why visual aids (Pictures, diagrams, charts, and films) are such valuable aids to teaching and why practical demonstrations are essential.

Sound

It plays quite a part in communicating a message too (a whistle, a car horn, a bomb blast military, music a love song)

Actions

Speaks louder than words, so it is said and it is often how he reacts physically, conveys a clear message both to the communicator and the receiver.

Mannerisms

Covers facial expression and other behavioural signals (a smile, a nod) to show understanding, a frown or head scratching to show lack of understanding. Look for mannerism when to communicate with a receiver.

Taste, Smell & Touch

All convey message to the receiver. For example there is no sugar in our today. The bearing in that machine smells hot.

Silence

We sometimes forget that we can also communicate through silence. This could show deep understanding of a speaker's words and unwillingness to interrupt him, but it could also show our strong disapproval of what he said.

BARRIERS TO EFFECTIVE COMMUNICATION

There are many barriers which makes it difficult for the communicator to reach his goal. Some of the important barriers are;-

I. Age
II. Socio-economic status
III. Language Vocabulary
IV. Attitude- Senders or Receivers
V. Competition for attention

I. Age- The age difference between the sender and the receiver

These age barrier almost always exist when two people of different age brackets are communicating with each other. A common language has to be found that will suit both the sender, who may happen to be a sophisticated middle-aged executive and the receiver and upcoming recent college graduate.

II. Socio-economic status

The economic gap between two different income groups is another barrier. For example a two car owner is not communicating when he tells someone who cannot even afford to take a taxi hat safety belt should be installed in the car.

III. Language Vocabulary

Barriers exist when the sender uses technical language to a group of laymen who have had no previous experiences in the area.

IV. Attitude- Senders or Receivers

Men of different political persuasions belonging to different schools of thought of political ideology or parties seldom communicate effectively of influence each other.

V. Competition for attention

There is much competition for attention of the audience at any given time. To be an effective communicator, you must be able to catch the audience's attension and hold it throughout the period of the presentation. A good communicator should recognize and

remove all communication barriers before starting to communicate.

HOW TO OVERCOME BARRIERS

The sender must know his audience's

- Background
- Interest
- Language

As a communicator you must remember that your audience is a human not a machine or mere cogs in a machine. Every audience is unique. Each responds to the sender's message according to his/her own drive, attitude, motivation, his education or lack of it, his social background and his interests.

As a communicator, it should be remembered that knowing your audience is not as simple as it sounds. One must understand human nature and human needs. Study your audience thoroughly and use language that your audience understands in order to minimize communication barriers. Secondly the message must be timely, Meaningful and applicable to the situation.

COURSE OBJECTIVES

The students should be able to understand and define various health records indices

A health record index is a selected listing of patient's health information representing more
comprehensive health records of the patients. It includes:

Master patient index

Operational index

Waiting list index

Physician index

3.2 Master patient index

This is defined as the alphabetical key to the numerically filed case records

It should be used to establish whether a patient has previously attended the hospital or group of
hospitals, if a group index is in operation. This is the section of the medical records department where most concentrated activities take place

In large hospital with 700 beds and say 200,000 outpatient attendance a year as consultative clinics,
there are likely to be as many as 300/400 references made each day and about 25,000 new cards
created annually. These means the index should be accessible to any those people with a legitimate need to consult it, access must be fast and facts on the card must be clearly
displayed

3.3 Filing equipment for master patient index card

Equipment chosen should meet the essential criteria for a master index i.e. access must be fast and
clear display of facts. There are other factors that should be considered when making a choice of the
equipment to be used e.g.

The quantities of records involved

Frequency of access

Space available for installation

Money available

i) Cards

This forms the basic record document for master indexes

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A card used for this permanent record must therefore be of good quality for endurance but not too

thick as to make the index expand too fast

Cards will be bundled many times when the index is being consulted and therefore be carefully be

guillotined to avoid fraying

I t is good practice to have cards feint ruled to keep handwriting neat. It also helps to leave sufficient

space for subsequent change of address

ii) Filing drawer

Here the cards would be blind .that is one behind the other in an upright position. It is good practice

to have drawers constructed specifically for the purpose of a master index since the master index will

be continually expanding forward and therefore planning to provide sufficient drawer space is

necessary to hold cards that may be created over a long period e.g. 80-90 years.

In most medical records, department have run into space problems simply of not planning ahead

even as far as 5 years

iv) Strip index

As the name suggests it consists of a series of this wooden strips coated with paper on which the data

required for the index are typed or handwritten. The strips are so narrow that any typing can provide

satisfactory reading

Strips are usually filed on metal panels with a fixed of metal down their edges which holds the strips
in place
These panels are then hung on a central revolving spindle so that they fan out like the leaves on an
open book
They can be wall mounted on a modified version in a book form
While the strip index provide excellent visibility as a master index, it has serious
Limitations in that it cannot handle more than 2,000 new cards created in a year. It is simple to
operate but with expansion the panels become foil and bulky too soon as new cards are added onto
the index
Within no time and due to constant handling strip lose their elasticity and drop out of panels
The disadvantage is that there is limited space allowed for writing details on the strip usually
measuring "¼"by 8 and these leaves no space for change of address
The strip index is however ideal for the record of patients in a ward or hospital
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v) Elevator files

Cards are stored in trays which are accommodated in racks centrally fixed so that the tray with the
required cards can be brought to eye level with the press of a button in an alphabetical display panel

vi) Circular carousal

This consist of shelves on suspended files rotating on a fixed spindle which may be fixed on a wall
which can be rotated to bring the files that are needed to the front

vii) Computer

The master patient index is presented as a computer view on the monitor

The keyboard is used to alphabetically identify they required name(s)
The method allows many to access the master patient index at a time

3.4 Procedures in filing patient master cards

The following procedure will be of assistance in patient master index and filings

a) Write the name of the patient placing surname first followed by middle name and others in
full and filed in strict alphabetical sequence

b) Arrange the card in an alphabetical order like words in a dictionary following letter by letter
to the end of the name

c) There should be only one card given to a patient if a unit system is used

d) Compound or hyphenated names are filed as one word

e) Name withy religious titles such as brother, pastor are filed last or the title disregarded

f) The legal name of a married woman is the one considered for filing. In this order a husband's
surname (Mr.) is placed in parenthesis i.e. [ ]

g) Only one person should be assigned the responsibility for filing the master index cards and is
in charge of other personnel's

h) The card should be checked periodically for misfiling

3.5 Diagnosis indexing

Definition

A diagnostic index is the process of bringing together all details of patients suffering from the same
disease in an index card.

The primary purpose of this index is to provide access for study purposes of clinical date contained
in the hospital medical records such that if a researcher wishes to study any particular disease at the

case folder numbers of the cases he wishes to study can be obtained from index and case records
extracted from the library for his study
It also serves as a means for the production of statistics pertaining to the incidence of various
diseases and injuries and any other factor about the patient that is recorded.
A diagnostic index is a standard feature of medical record system of most hospitals in the world
particularly in reaching the hospitals and those hospitals where many clinicians and other
professionals are engaged in research
Equipment used in disease (diagnostic) index
Disease index can be created on
• Vertical index cards filed in drawers
• Visible edged cards filed in pockets
• Margins punched cards
• Slotted punched cards
The choice of equipment to be used depends on the volume of records to be dealt with and the
amount of information to be recorded

3.6 Operational index

Besides the diagnostic index the health records department has to maintain an operational index
Operational procedure need to be coded just on diagnostic according to ICD of classification of
procedures in medicine

Equipment used to maintain operational index are the same as those used for diagnostic index

3.7 Waiting list

Definition: This is an index of patient waiting for an admission in the hospital or

patients waiting for
treatment on a day case
Most of the waiting list records are initiated at outpatient clinic. Some other patients may come from
another hospital and be put in a waiting list in another different hospital
There are four ways on how messages can be conveyed to the waiting list

Card

The card is created for a patient who is to be put on a waiting list. This card will be filed and forms
part of the waiting list

List

The nurse or a doctor may send a list of patients to be included in the waiting list to the Health
Records department for action

Letters

A consultant in one hospital may wish to include his patients on the list in another hospital
The information contained on the waiting list letter
Address

Title

Name of the patient
Telephone number
Holiday's days

Diagnosis

Operation to be carried out

3.8 Functions of a waiting list

The waiting list must be organized so that inquiries may be answered from individual waiting to
know when they are supposed to be admitted in the health institution
Individual consultant who would like to know how many patients are on their waiting list should be

furnished with some information

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3.9 The main function of this list is to be able to make full use of the available beds in a

health institution

Types of waiting list

Centralized waiting list

Decentralized waiting list

3.9.1 Centralized waiting list

In this way the waiting list is held in one office and contain names of all patient awaiting admissions

for all consultants in the hospital

Advantages

Gives a fair representation of all the demands being made on the inpatient facilities

All inquiries are referred to in one place

Staff dealing with the waiting list develops some skills in dealing with inquiries

Updating procedures such as change of address, date are carried out easily

Checking admission and discharge from the daily returns is easier

When one staff member falls sick or on leave other staffs can carry on with the work

Training of the staffs becomes easier

Disadvantages

Consultants need to walk to the central waiting list to select their patients from the waiting list

The list becomes so big and long that some patients may be left out of the waiting list

3.9.2 Decentralized waiting list

This would be maintained in several places possibly by each consultant's secretary or by individual

wards or departments

Advantages

The list is short and easy to maintain

Consultant need not to go to the central office to select their patients from the waiting

list

The secretary gets familiar with the patients and can call them by their names

ELECTRONIC MEDICAL RECORD SYSTEM

Definition:

Electronic medical records about patient care that are kept on a compute rather than on paper, the tradition medium for patient histories. Those records can include extensive information about patient's general health, current and past illness and medical conditions diagnostic results and treatment and medication prescribed. Often electronic medical records also include an application for prescribing and ordering medication.

Medical records ensure that patients receive the best medical care possible and that they are kept safe during treatment. Traditional paper mean of medical record keeping can be burdensome to maintain and track causing electronic medical records to come into safer. Knowing what electronic medical records are and how they work can help a medical clinic or hospital determine i.e. the implementation of an electronic system would be beneficial.

The medical industry uses electronic patient's records as a mean of establishing more convenient and efficient patient records health care offices and hospitals can integrate computers into their clinical practice.

Electronic patient records are clinical records that are stored electronically in a computer based system according to open clinical patient records house various types of information about a patient record even have technological capabilities to track patient billing information and keep track of patient appointment.

An electronic medical record is any medical record that is kept in electronic format. They contain the same information as traditional paper medical records (e.g. patient name, condition, medicine prescribed) but the data is not written out longhand. Instead it is entered into some type of electronic device such as a computer or PDA the data can be retrieved by these devices later when needed.

CREATION

Electronic medical records most commonly are created on computers. The computer is set up with some type of records software and a technician enters data into the fields in the software to save it into the system. Most types of medical record software have the ability to search for particular data such as patients' last name or insurance number. This will help to retrieve data very quickly. EMR also can be created by sophisticated medical devices such as machines e.g. that monitor the heart it sends data to a server during a stress test. Due to advances in the technology, they even can be created through application on cellular phones.

AUTHENTICATION

As with tradition medical records, technicians still need to verify that the information contained in electronic medical records is accurate. EMR must be unaltered and be authenticated in order for them to be accepted for use.

BENEFITS

EMR greatly reduces the amount of space needed for record storage. They also decrease the amount of time that a must be spent physically retrieving files. They can be transmitted over a long distances in a matter of seconds, can be more legible than handwritten records. Lastly electronic records speed up billing and inquiry processes which means the hospital is able to serve more people and provide better services.

EMR alleviate some administrative burden for medical officers because they eliminate the need for documents to be billed and filed. This saves medical professionals time, which can be better spent with patients.

CONCERNS

EMR are prone to data entry errors. They also are sensitive to data corruption if an electronic record is lost and there is no paper record, there may be no way to retrieve the data. EMR keeping also means that those who create and use the records must be comfortable with the related technology. Cost may rise for the hospital due to their training requirements, the purchase of necessary equipments and technology support.

FEATURES

Medical professionals can enter new patients into their electronic records system easily, as well as change and update pre-existing records. Records can be deleted when patients are no longer a part of that doctor's office, they can be faked to specialty providers for medical collaboration and copies can be printed for patients to take home.

ADVANTAGES OF EMR

1. The data tends to be more accurate. Electronic records eliminates the possibility of mistakes as a result of misleading a doctors hand writing
2. They are easily accessible to all care providers and more than one care providers at s time

3. They are easy to store and take up less space than paper records
4. They are easily portable form one doctor's office to another
5. Their use can lead to cost saving since keeping electronic records is more efficient than retaining paper records.

DISADVANTAGES

1. The possibility of incompatibility of computer system among various health care providers can lead to difficulty in sharing the data
2. Privacy and security cab be an issue, if someone hacks into the system, thousands of patients records can be compromised.
3. When a computer crashes makes records inaccessible
4. The cost of implementing an electronic records system can be expensive

OBSTACLES

Cost remains a major deterrent to the implementation of electronic medical records, especially in small practices. The initial outlay for these systems can cost hundreds of thousands of dollars, something many physicians are not willing to spend. Doctor's reluctance to accept the new systems has also slowed their adoption in the United States, according to a journal of American Medical Association

NUMBERING AND FILLING SYSTEM

INTRODUCTION

Medical records in most health care institution are filed numerically according to patients' medical records numbers. In the past some hospitals have filed records according to patients' names, discharge numbers, or diagnostic code number. Alphabetic filling by discharge numbers are more cumbersome and subject to more errors than numerical filling. Filing by discharge number and diagnostic code number is generally unsatisfactory because other important records or registers in the facility are concerned exclusively with medical records numbers

1. **NUMBERING SYSTEM**

There are three types of numbering systems that are currently in use in health care facilities,

they are;

i. Serial Numbering system
ii. Unit Numbering system
iii. Serial unit Numbering system

BED BUREAU

Most of the information contained so far in this section is related being admitted from the W/L, although similar steps have been taken when a patient is admitted as an emergency. The bed bureau is the section of the medical records departments arranges admission for emergency patients. The functions of the bureau is to receive the telephone calls from the GPS who wish a patient to be admitted in emergency, To pass the information to the accident and emergency departments or to whichever department which receives the emergency admission to initiate the retrieval of the old (notes) case records.

If one exist the creation of a new one if the patient has never been to the hospital before and most importantly to locate the bed to which the patient may be admitted in order to be able to do this. The bed bureau staff must have an exact and up to date knowledge of the bed state in every ward, in the hospital. Thus if a GP telephone about a patient whom he believes is suffering from acute appendicitis, the bed bureau will look to see which surgical ward has empty beds and telephone the ward with the details of the patient whom they may expect to

admit later in the day.

The actual decision as to whether a patient is finally admitted rests with the doctor, who examines the patient when he comes to hospital. The bed bureau as mentioned is a method by which it can keep up to date with the bed state of the hospital. This board may consist of a series of looks, slotted racks, peg hard .etc. divided into as many sections as there wards within each section will be as many divisions as there are beds in that ward. Each section is headed with the name or number of the ward and the number of beds. It contains when a patient is admitted something will be put into its slot or onto the look to represent that patient, in that ward. Using this card in the slot, will give the patients details indicating the occupied bed.

If a 25 bedded ward has 20 slots it will be obvious at glance that there are 5 empty beds in that ward. However it is advisable to be able to see how many waiting lis beds are expected in during the next day or two or a ward with 10 empty beds may be expecting 10 patients from its waiting list next day. Whether a ward with 5 empty beds may not be expecting any waiting list admission. Wherever is used to represent the patient should be put into a special section for expected admission so that the bed bureau staff may make appropriate arrangements and not to overload any single bed ward while another has several beds empty.

Other bed board is kept up to data as follows when the daily ward statement is received the cards in the board are checked to see that they agree with the number of patients stated to be in the ward as each patient is admitted to hospital during the day a card is put in to the bed board. Immediately the bed board staff may visit or telephone each ward at interval during the day to see what patients have been discharged or transferred to another ward. After each visit or telephone call the bed board will be attended immediately. Thus at the end of the day the bed boards should represent accurately the number of patients in each ward and give the staff who will be responsible for admissions during the night a guide as to where the empty beds for these emergency admissions can be found.

A bed bureau can save a great deal of medical and nursing time in locating empty beds in emergency admissions. They can help to make bed use what is one of the hospitals most expensive item. The inpatient bed and assist GPs and hospital staffs to arrange emergency admission as quickly as possible. A bed bureau may act for one large hospital or a group of

hospitals, if it is dealing with emergency admission to several hospitals some of the procedures suggested would have to be modified but, the general principle would be similar. The key note of success is good liaison with wards, GPs Accident and Emergency departments' admission office and the medical records department. All these sections are involved in some parts of the admission procedures for emergency patients and must be aware of the functions of the bureau.

Location of a bed bureau

It is possible to combine the function of admission office and the bed bureau so that the same are dealing with waiting list and emergency admission. In this instance, the office must be reasonably near the ward which is also convenient when staff make their visits to the wards. It may however be situated some distance from the wards. Accident and Emergency department and admissions office and still function satisfactory provided liaison and good personal relationship with all the departments concerned.

DISCHARGES PROCEDURES

Just as a sequence must be established and followed for all admissions, so the sequence will be needed for all the various steps that have to be taken when a patient has been discharged. The medical records department is likely to be involved in the following aspects.

DAILY WARD STATEMENT/ WARD REFORMS

1. **Layout & content**

Daily ward statement is a return made by each ward at the same time each day relating to the movement of the patients in to the ward and out of the ward. Some hospitals check/take the daily counts at 12.00 mid night and others at 8.00am. The advantage of an 8.00am count is that all the nights emergency admission will be included and various departments convened will automatically have more up to date information at the start of the working day. Whatever time is chosen it must be the same time for all wards in the hospitals and the time of the count must appear on the form itself the statement made in the separate pile of paper, one being completed each day.

There are advantages however in long term bound in a book form (Duplicate), with one removal top copy and a first, second copy completed by carbon paper. The whole book may be taken to the office and typing the statements or the top copy and sent in either cases. The copy of the figure return one day will be there to guide whoever is completing the return the following day (The returns because of the time concerned are invariably completed by nursing staff). The statement must include the following information;

i. Headings at the top of the form giving the time for completion and space to fill in data and ward concerned

ii. Sections for patients admitted i.e. patients coming into the wards from home or from another hospital

iii. Section for patients transferred in i.e. patients moving in from another ward in the same hospital

iv. Section for patients discharged i.e. patients leaving the hospital either to go home or to another hospital

v. A section for patients transferred out i.e. patients moved to another ward in the same hospital

vi. A section for patients who have died in the wards.

Each section must make provison for the patients name hospital number and the name of the consultant in whose care he is. In addition some hospitals have a column containing the names of all consultants who have beds in the hospital and ward has to enter the number of patients which each consultant have in that particular ward. The total at the bottom of the column will naturally add up to the number of patients in the ward. Finally there is a section at the bottom of the page summarizing the actual bed state headings are likely to be;

- Number of patients in last return(s)
- Number of admissions and transfers in
- Number of discharges and deaths

- Number of patients remaining

The number of patients remaining should agree with the figure shown at the end of the column. In which patients are analyzed by consultants and also with the number of cards in the bed board. Hospitals may also use the daily wards statement. The number of private patients in the hospital and the number of well people lodged in the ward (i.e. relatives who have been accommodated over-night.

STATISTICS INCLUDING H.A.A

The daily ward statement provides figures that are the basis of hospital economics and the management as far as inpatients are concerned. It is the sorce document for many financial and administrative that gives the statistics on which planning of resources will be based e.g. The number of beds that are likely to be needed in any future development of patients in a certain speciality. It can indicate the use which is being made of the beds for examples wherether a high occupying rate is being achieved or whethere beds aare lying empty for several days before the next patient is admitted. It is eveident, therefore that 100% accuracy must be achived each day. The return must balance. The medical records officer will probaply arrange that the admission steps for the patients admitted during the provison (24hrs) are checked against the daily statement to see that there is a slip for every patient whose name appears on the statement and also to see that there is a name on the form for every slip held.

This can be doubled checked by making sure that the bed boards take the same story as the ward statement. Only then can the statistics that are needed be extracted. It is important to know the background that will provide the in-patient statistics, where hospital is fully committed to carry out hospital activity analysis the ward statement will play an important part also.

DISCHARGE LIST

The same section that wishes to know when a patient was admitted will also know that he was (has been) discharged. List of patients discharged will probably have to be prepared to notify at laest reception clerk on the telephone switch board and of course the medical records department. It is possible to pass the actual ward statement to these section so that they may remove the admission slips of those patients who have been discharge or died and amend the ward for the patients who have been transferred. If computer documentation is used, it is possible to prepare a discharge list and print out those discharged or died patients so that the daily ward statement will not leave the medical records department.

FOLLOW UP APPIONTMENT

A. INITIATION BY WARD

Each hospital will have its own way of amking follow up appointment for patients leaving the ward. It is most satisfactorily if the appointment card can be given to the patient before he/she leaves the hospital bed this may always not be possible. The wards should hold stock of special request forms on which appoitments can be requested. One of this forms plus the patients appointment card should be sent down to the appointment section preferably the afternoon before the patient is being discharged. If it is anticipated that the patients will used hospital transport this should be ordered at the same time.

B. CHECK BY WARD CLERK/DISCHARGE CLERK

Whoever is responsible for putting the patient list in order after he has left the ward should check that if a further appointment is needed it has been made.

COLLECTION & PROCESSING OF NOTES

It is desirable that there should be point in each ward from which the notes of patients who have been discharged can be collected. If there is a ward clerk his desk is the obvious place, otherwise it may be the sister's desk of office.

A. COLLECTION BY DAILY WARD STATEMENT

The medical records department may be responsible for collecting notes and x-ray films from the ward, if so the clerk concerned should take with her/him the daily ward statement and check that she has a set of notes for each patient who is no longer in the ward. This is to safeguard against notes being retained in errors so delaying further process or procedures.

B. COLLATION

The notes should be put in order by the ward clerk but they frequently arrive in the wards department with many loose papers. The clinical notes investigations and reports relating to the admission are to be filled in the agreed order in the folder and any transitory documents that do not have to be retained by law such as 4 hourly temperature charts may be discarded at the stage of that is the hospital policy.

C. DISCHARGE SUMMARY

The ordered notes are then taken to the medical secretary who will be responsible for typing the discharge summary of formal report to the patients G.P

D. CODING

When the discharge summary has been dictated and typed the notes now containing a list of diagnosis made while the patient was in hospital will be passed to the coding and indexing section so that the relevant details may be extracted to form the disease index. Coding may be carried out by a trained clerk. It is at this stage that forms for hospital activity analysis and the hospital inpatient will almost certainly be completed. The coding clerk will also note that diagnosis which relate to malignant diseases and either pass the notes or a list of names and hospital numbers of the patients concerned to the follow up department as indicated in the section dealing with follow up procedure.

PREPARING OF A CLINIC

Two days prior to the clinic list should be sent to the following sections,

- To the clinic

- Filling area (Library)
- Consultant incharge of clinic and deopy left in the registration area (office).
- All the records and documents should be pulled out, the pathologic reports inserted in the correct files and kept in the pigeon hole for certain clinic to come and collect.
- The person running the clinic should make sure that all the documents such as x-ray, lab results are filed (in the patients file)
- The clinics are prepared 24hrs in advance to reduce the patients waiting time before he/she is seen by the consultant.
- At the end of the clinic the personnel (HRIO) should ensure that all files are returned to the library for filling

Waiting list for in patient and day cases treatment.

Define: A waiting list is an index of all patients waiting admission to hospital or waiting treatment on a day case basis.

Functions

- Is to provide a list or an index of all patients awaiting admission to hospital for treatment in any given day or on a day case basis
- The list must be so organized that it can immediately provide answers from;

i. Individual patient enquiring about their status on the waiting list and likely their days of admissions.

ii. Consultants wish to see how many patients they have on their list and what priority they have been given for admission.

-The main function of waiting list is to be able to make full use of the available beds in a

health institution.

-In this parts, the consultants can be able to arrange admission in a way that its convenient to the patients and hospital (institution) from the point of viewing of the making up a balancing operating lists.

-A waiting list may be maintained on centralized basis or decentralized basis.

CENTRALIZED WAITING LIST

Centralized waiting list is held in one office and contains all the names of patients awaiting admission under all the consultant in the hospital.

Advantages of Centralized waiting list

-Gives a fair representation of all demands being made on the inpatient facilities.

- All enquires are referred to one place.

- Staffs dealing with the waiting list develop skills in dealing with enquiries and in the maintenance of the waiting list (deal continuously with the waiting list as the sole task. This is necessary to maintain a large waiting list in good order)

-Updating procedures such as change of address, death are easily carried out and less time consuming.

- checking admission and discharges from the daily returns is easier as only one person has to carry out this task.

-When on staff falls sick or goes on leaving another staff can carry on with the work.

- Training of waiting list staff will also be easier and faster as all new staff members will be trained at one point or in one place.

-Less staff members are required

Disadvantages of Centralized Waiting List

-The list becomes so big so that some patients may be left out of the waiting list.

-Consultant needs to walk to the central office to select their patients from the waiting list

DECENTRALIZED WAITING LIST

The decentralized waiting list will be maintained in several places possibly each consultant secretary, or are individual wards or individual departments.

Advantages of Decentralized Waiting List

- The secretaries get familiar with the patients and can call them by name.
- The consultants need not to go to the central office to select their patients from the waiting list.

CREATION OF THE WAITING LIST RECORD

Most of the waiting list records are initieated at the out patient clinics. Some other patients may come from another hospital and be put in the waiting list for another different hospital, where there are more beds or facilities. There are four ways in which information can be conveyed to the waiting list.

a) Cards

b) List

c) Letters

d) Filling

A. CARDS

-Supply of cards should be kept at the clinic

-Cards should be designed such if mechanical documentation is used, it may be computed partially by the used of self-adhesive labels giving all the patient social data.

-The cards may be single cards or they may be put together in book forms so that the cards used may be toned off.

-With this make up it is possible to name counter fall so that clinic maintains a list of patients. These patients whose names have been added to a waiting list in case of future questions.

-The records will be usually created by the doctors writing in the medical details and then passing them over to the nurse or HRIO for the completion of the social data on the card.

Note:The card is created for every patient who is to be put on the waiting list. This card will be filed and form part of the waiting list.

Advantage

-Its stiff and not easily lost and can in its original form used as part of actual waiting list file.

B. LIST

-The clinic nurse or the doctor can combine a list of those patients whose names are to be put on the waiting list. The list can be sent to the waiting list office for further action.

Disadvantage

-The list itself will not be suitable in any part of the waiting list. Information on the list will have to be transcribing which will involve a list of clerical work which may lead to errors.

C. LETTERS

A consultant in one hospital may wish to include his patients name in his waiting list, in another hospital for his name to be included in that waiting list. In this letter he will include diagnosis and priority for admission. A case folder will then be created for this patient. The information to be included in this record are;

- Patients name
- Address

- Title
- Telephone number
- Holiday dates
- Diagnosis
- Operation to be carried out
- Duration of stay in the hospital
- Name of the surgeon/consultant

Note: All patients put on waiting list must be fully registracted and have a unit number. If patient is admitted as an emergency, the folder will then already be in the file with the relevant information.

Drawings

Waiting list records

Waiting list request form/letter

CONTENT OF A WAITING LIST RECORDS

Two types of information must be contained on a waiting list records

1. Social data
2. Medical data

SOCIAL DATA

This include;-

- Patients title

- Name
- Address
- Telephone number
- And where possible such details has holiday dates & minimum noticed required for admission.
- This is specifically important for example when a mother have to make arrangement for her children to be looked after bu someone else or when a man may have to make arrangement for his business to be looked after by someone else.
- A school teacher may prefer to be admitted during school holidays
- Some patients may be willing to come for admission at short notice and it is important to have a reserve for such patient so that if a bed becomes an expectedly recent someone can be called in who is willing to come without several days delay.

MEDICAL DATA

This will include;-

- Diagnosis/operation to be carried out prior of admission e.g. routine, agent, soon and indications as to whether the operation will be carried out by the consultant or register. Some institution will indicate how long the patient may be expected to stay in hospital. Therefore when a patient is to undergo an operation is admitted it should be possible to get when he will be discharged & when his bed will be available for another patient. A statement is also included as to whether or not a patient will need hospital transport to bring him to

hospital

- It's important to have full decision with the patient. This is such an important event for the patient then he will wish to clear the situation made quite clear by the doctor.
- It's also important that the other details required are carefully recorded at this stage.

METHODS OF FILLING

A Waiting list contains dozen names and needs two files;-

i. Alphabetical Index- of the names of all patients on the waiting list. These cards should not be removed until the patient is admitted.

ii. Consultant list- Will make provision for date, time and ward. This list will have priorities. Indicated – routine soon and urgent. The two files should be able to answer an enquiry from a patient from a patient and consultant.

Equipment used in the waiting list

-The types of equipment chosen for the maintenance of the waiting list will depend to some extend on the anticipated size of the list. A hospital with 4000 patients awaiting admission will have to make different arrangements from one with 400 patients on the list

-The longer the patients have to wait on the waiting list for admission the more important it is that there should be space for expansion

STRIP INDEX

- Consist of the wooden strips coated with paper on which the details required for the waiting list are typed or written

- The strip is so narrow that only typing can really be neat enough for satisfactory reading
- Are filed on metal panels which hold the strips in place. The panels are in turn hang on a central removing spindle so that they can fan out like the leaves of an open book.
- They may also be wall mounted or in a modified revision in a book form
- The strip index provides excellent visibility. It has one serious draw back which is difficult in expression. It's relatively very simple to add a new strip simply by pushing the strips above the place of entry up the metal panel, inserting the new strip and then moving the strip down
- However as new ones have to be added, the installation can be very bulky, due to constant handling the strips tend to lose the elasticity and to drop out of the panel easily
- Limited amount of space for writing. It usually measures ¼' by 8' and is not practice to carry out a change of address.
- Information recorded on the strip would include the patients name, date put on waiting list, diagnosis, operations admission priority and space left for an insertion of a date of admission. Due to the limitation in space for writing information that is necessary for the proper maintenance of waiting list this form of waiting list where patients may be waiting for several months and years before admission. There is no space to record change address and more than one admission data.
- Nevertheless, strip index has a useful application on the keeping of a waiting list that is not too large and should be considered with other possibilities.

SUMMARY OF STRIP INDEX

-Is limited in space so that no space is left for change of address

-It can only be used if the list is not tool long

-It has already been mentioned as one of the equipment used in the maintenance if the patients master index cards.

VISIBLE EDGE CARD FILED IN TRAYS

- When this is used to maintain a waiting list may be in the form of manila flaps with a rigid bar at the top and a transparent plastic pocket about an inch deep at the lower edge of each flap.
- A card is inserted into this pocket and the flap is held in a metal tray in such a way that all the information in the plastics is visible. Data is put on the list, diagnosis, operation and admission priority is put to allow the consultant to select his patients.

DIARIES/BOUND VOLUMES

- A centralized waiting list cannot be kept in a diary form but it's possible that a department all list containing relatively few names could conveniently be held in this form.

Advantages

- No page can be closed until every remaining entry has been dealt with

Disadvantage

- A single entry remaining or a page that is otherwise finished which may be overlooked on date passed.

COLOUR CODING/SIGNALS

As well as marking the correct priority on each card it is helpful to add colour to indicate the degree of urgency, soon urgent etc. Small plastic cards can be inserted in the pocket of visible edge cards to emphasize that the card has some form of urgencies.

PROCEDURES FOR ADMITTING PATIENTS FROM LIST

- The patient selected from the waiting list by the consultants or by his registration, he will either write the date of admission and ward on the card or instruct the waiting list clerk to do so. As much as notice as possible should be given to patients and the medical staff should choose as far in advance as possible.
- The clerk writes to the patient or telephones with inviting the patient to the hospital or as last resort only ne sent a telegram.
- The records are got out from the files and sent to the documentation office. Where necessary in patient forms will be prepared by either mechanical documentation or by hand. Preparation means completing the patient identification details in each form and adding such details as date of admission, consultant, ward etc. The following forms may be prepared in advance;
    - ✓ In patient clinical records sheet
    - ✓ Surgical operation record

- ✓ Anesthetic record
- ✓ Temperature chart
- ✓ Prescription record/ Treatment sheet
- ✓ Discharge summary form
- ✓ Labels

- The waiting list card is sent to the admission office so that the patient is expected on the day ne comes in. The admission office checks the record before the patients comes. When he/she comes his details are checked and confirmed by the admission office.
- The patient is admitted and sent to the ward
- When the patient has been discharge his name is removed from the waiting list
- Certain checks are made on the waiting list to remove the names of the patient who have died to remove their name form the waiting list.

CHECKS TO BE MADE ON THE WAITING LIST.

The facts recorded in the waiting list must be correct;

i. The names of all patients for whom admission are planned must appear in it,

ii. Names of all patients who no longer need admission must not appear on it. Extreme care has to be taken in each item of medical records always remembering each card or piece of paper of folder represents a person who is in and relying on the hospital to see that the he has the best chance of getting better. The following checks are suggested as being some of the

ways in which the MRO can ensure that the list is fulfilling its purpose of displaying in the agreed order of the names of patients reading admission.

a) Checking for completeness.

-The methods by which patient names may reach the waiting list have already been described. It's possible however that a particular name may be missed, the doctor may think he ask the nurse or the receptionist to make a card but may not have done so.

- The hospital may send a letter for the GP (Dr.) saying that an admission is planned but unless the Dr. has a very clear idea as to when the patient is going to be admitted he is likely to follow this information up. The patient will wait believing that his name is on the waiting list and he will be admitted in due course. A failure to enter the patient name on the waiting list, therefore would lead to grave delay and prejudice the patient chances of recovery.

b) Checking for Accuracy

-Where a hospital has a large waiting list of more than 1000 waiting for admission, some of these patients will wait for a long time.

-It is important to be certain that the patients at the list are still in need of treatment and have not moved from the area or had the operation carried out a another hospital.

-It is necessary in order to make this check, to write periodically to all patients who have been on the list for more than 6 months or year. The letter must be carefully worded explaining that the list is being checked and asking if the patient still needs to come in to the hospital. The patient should also be asked if he feels that for any reason admission is no longer necessary. A business reply or stamped envelope should be included with the enquiry. Most patients will reply saying that they still wish to come in some will have had treatment elsewhere, while some say they no longer wish to come in. The replies of the letters in two categories must be referred to the consultant as he will wish to write to the patient. GPs before taking the patient names of the list. These

correspondences should be filed in the patients file.

Note: This check for accuracy means that patients who no longer need admission are not sent for and that the waiting list statistics are not wrongly inflated. It also reassures patient that the hospital has not forgotten about them.

c) Checking for duplication

-The admission forms of routine and emergency admission should be checked against alphabetical file. In this way the card of a patient who has been in the routine waiting list for the repair of hernia will be removed. If this patient is admitted as an emergency with a strangulated hernia. Similarly if a patient is admitted with bronchitis and his name is on the waiting list for removal of tonsils the card relating to the tonsils should be marked admitted with bronchitis and date so that it cannot be sent for to come to the hospital for the operation when it is already an inpatient in the hospital.

d) Checking for Deaths

-The deaths certification procedures must be carried out with the death notification form and sources. The daily ward statements will be one of those statements and a "died" form should be initiated by whoever is responsible for checking the ward statement each morning. Nothing could be much more upsetting for relatives than to receive a notice for admission of someone who is dead especially if he has died in the medical ward of the very hospital that is now asking him to come into a surgical ward. But this can happen unless deaths are checked carefully against the alphabetical waiting file and the cards of all patients who have died removed immediately.

STATISTICS

Regular returns will be needed of number of patients on the waiting list. These returns are usually in the form of number of patients waiting for admissions under each consultant male,

female, children may be shown separately. The number of patients waiting for more

than 6 months may be given.

-Hospital carrying out hospital activity analysis may receive the statistics as a by-product of the information they have provide when a patient is put on the waiting list.

-Most of the procedures described will apply for day case treatment. It's common practice for day case patients to be put on the waiting list and called the same day. Day cases should usually be called in using special letters because of the preparations that may be necessary if the patient is to have general anesthesia is to be given.

IN PATIENT PROCEDURES

-It's important that arrangement for patients' admission should be clearly defined and known to professional and lay staffs working in the areas concerned i.e. clinic, wards, waiting list, bed bureau, A&E etc. The patients G.P should also know what arrangements are involved when patients are to be admitted both in emergency and routine.

- The local ambulance services must also be aware of arrangements that need to be made for different categories of patients whom they may be bringing into the hospital.

-If all these parties are fully aware of admission procedures, the patients will be admitted to the hospital with maximum speed efficiently and less trouble to the patient.

PHYSICAL REQIUREMENTS FOR ADMISSION OFFICE

-The admission office should be as near to the entrance to the main wards as possible, but should also be near the main entrance of the hospital so that patients brought by means. Other than hospital means will easily find their way.

- Sign posting should be clear and unambiguous in the letter ensuring the patient to come to he hospital. The office should be described by the same name that appear not be asked to report to the admission office if sign posting to if records in patient reception office.

- There should be good provisions of confortable chairs one magazines. If patients are likely to have to wait before being escorted to the ward. Flowers and pictures will make it cheerful and welcoming. Most important of all these should be a receptionist on duty at any time patients are likely to report for admission. The receptionist should have a list of patients who are expected and know what to do with minimum fuss e.g. of a patient arrives who she is not expecting.

PROCEDURES

A. PROCEDURES RELATED TO PATIENTS

- The waiting list office will have sent down full information about patients who are expected from this a admission form will be prepared in the admission office. The admission for will usually be a multiset and will contain patients social data including next of keen and telephone number at which his relative would be contacted. This is the information contained in the pre-registration slip which forms part of the letter to the patient.

- The admission form will also have date of admission the ward to which the patient will be admitted and name of the consultant in whose care the patient will be.

- Time of admission may be useful especially on emergency admission. The admission form should be completed in advance wherever possible.

- When patient arrives, details on the form are checked and any missing items are completed. If pre-registration has been sufficiently done it will barely be necessary to ask patients any questions but this is a good opportunity to have a brief conversation to make the patient feel welcome. The patient may be given information about the hospital visiting time etc. If he may not have had a question often asked at this stage " can I keep my clothes or must I send it home" The receptionist must have an answer to this and to many other questions that the patient may ask. These questions may seem and

sound very simple and familiar in the receptionist but they mean good deal to the patient.

- The patient should be escorted to the ward. It may be possible for admission office staff to accompany the patient or the pothering staff to accompany him. If both of them are not possible, most hospitals have voluntary workers who are willing to carry out this escort task. It gives patients a poor impression of hospital if the patient is left to find his way through what to him will certainly seem a maze of corridors.

  Note: Therefore someone to talk to on the way and help him with his case if necessary will make the journey to the ward less and ordeal. The patient should take with him to the ward a copy of admission form which will act as passport to the ward.

B. MEDICAL RECORDS PROCEDURES

- The multi part admission for already referred to the will need splitting in to several parts. These copies form an information bulletin that tells interested departments that the patients have been admitted. It is useful if different copies are coloured differently so that the ward e.g. has a pink copy etc. The departments that will need to be notified in most hospitals will include;

i. The ward

The top copy of the form has gone with the patient. The ward known by received are on the x-ray hence not been received are on the way. Wards must know that if a patient arrives in the ward without this form then admission office must be informed at once. They should be encouraged never to admit a patient without one.

ii. Records department

There will be several uses for this form. It may be used to check alphabetical index to waiting list to take out waiting card. It may then be filed on alphabetical order to form a list of all patients in the hospital an anytime hence taking place of admission register. This will facilitate enquiries forms should be removed and discarded on a patients discharge.

iii. Main Hospital Reception

- These files will be filled in an alphabetical order so that if relatives arrive and do not know which ward the patient is they can be divided.

iv. Medical Social Workers department

- Medical social worker department should also have a copy of this list that if any of their special patient comes in unexpectedly, they will be able to visit them as quickly as possible in some hospitals of social worker will automatically visit every patient who has been admitted in this case of the consultant whose patient she should look after.
- In other hospital social workers will visit only those patients on whose behalf a ward sister has sought advice.

v. The Chaplain

- Most hospitals will have resident or visiting clergy in the main denominations.
- There should be arrangements do that there are several kooks or pigeon holes where forms relating to patients of different denomination may be put. Some arrangements must be reached as to which chaplain will be given form of those patients with no religion.

vi. Telephone switchboard (Hospital)

- Hospital switchboard should have a set of their own so that any telephone enquiries can be dealt with appropriately and promptly. The records office should ensure a speedy dispatch of admission forms to relevant sections so that as many people as possible know what a particular patient has been admitted.

- Hospitals have to maintain a permanent record of those patients who have been in patient. There is no defining ruling however to state whether this must be an admission or discharge register provided the details of all patients are entered.

- The entry for each patient address, date of birth, religion, date of admission, consultant ward, date of discharge etc. There may be additional column or provisions, for recording only further information as when the notes were received back from the ward or whether the patient was transferred to another hospital/ or other information.

a) Entries are made in a chronological order i.e. name of each patient admitted is entered in sequence. Should this be the case, there would be an alphabetical index (Key) at the back of the register so that if an inquiry is made about a patient whose date admission is not known, the name can be traced up in the index and will give the page on which entry appears.

b) The entries are made in letter order i.e. name of all patients whose surname start with "A" are entered in one section etc. This is called a self-indexing register and on the type of register it is relatively easy to look through all entries under a certain letter until the correct one is found.

CREATING AND MAINTAINING HEALTH RECORDS INDICES

INTRODUCTION

The main health records indices described are;

- Master index

- Diagnostic index
- Operations index
- Waiting list index

Learning objectives

i. Define various health records index
ii. Explain the importance of Health Records indices
iii. Describe the procedures used in creating health records indices
iv. Describe the procedures used in creating health records indices
v. Describe various equipment used in storage of health records indices
vi. Describe the techniques used in filling health records indices

INDICES

There are various health records and information used and maintain a health records department.

The patient master index

Definition- The patient master index is the alphabetical key to the numerically file case records.

Also referred to master population index master person index. It's a permanent database including every patient are admitted or treated at the health facility.

Importance (Filling equipment to master patient index)

- Allow easy access and reference item included in it. (Table of Content)

- Helpful in settlement of social security claims, medical report without referring to patients notes.
- Master patient index is a source of material relating to patients. Information on master patient index should be sufficient to enable any inquiries to be traced.
- The master patient index should measure 3" X 5 or 2 ½ by 3" depending on available filling equipment.
- The master patient index should be kept as simple as possible having regard to this demands likely to be made upon it. The information contained on master patient index include;
    - ✓ Surname
    - ✓ First names
    - ✓ Sex
    - ✓ Hospital number
    - ✓ Address
    - ✓ Date of admission(DOA) and discharge (DOD)
    - ✓ Marital status
    - ✓ Gender
    - ✓ Next of kin
    - ✓ Space to be left for change of address

A typical layout of the card which measures 3" X 2 1/2 " or 5" X 3 " would be

3"

Name of patient	Hospital
No:	
	D.O.B
Address	Sex
Space for change of address	

2 ½”

Creation of A master

The master index card is created when the patient is being registered. Equipments used for filling the master index card are;

- Cards
- Elevators files
- Card wheel
- Carousel type files
- Guide cards
- Drawer filling

Drawer filling

- Cards may be filled “blind” in drawers. Cards are filled one behind the other in an upright position.

- It's recommended to have drawers constructed specifically for the purpose of a master index. Since the master index will be continuously expanding. Forward planning to provide sufficient drawer space as required to hold the cards that may be created over a long period e.g 20 years.This can be computed by getting the number of cards filed in one inch i.e. 80-90. If in a hospital the number of patients cards created in a year is 15,000 then the space needed for twenty years will be

$$= \frac{15,000}{85} \times 20$$

Inches for approximately 300 linear feet.

-In most case institution run into space problems simply because of lack of planning a head even as far as five years.

Cards

- Cards from the basic records document in master index. A card used for this permanent records must therefore be of good quality but not too thick as to make the index expand too fast. Cards will be handled many times when the index is consulted and should therefore be carefully placed to enable to frequent check.
- It is good practice to have card that can't faint and to keep handwriting neat and sufficient space left for subsequent change of address.

Elevators files

- Cards are filled in trays about a foot long and these trays are filed in shelves in the machine. The machine is a large metal cabinet which can be up to 5 feet high and 10 feet wide at the top. 2 or 3 of these shelves with their trays of cards are visible in the opening. The largest elevator file can hold as many as 900,000 cards of 2 ½" X 3 "index but you may get a smaller one with a capacity of 10,000 cards in the market.
- The larger machines can be driven electrically the smaller ones operate manually with the big machines the filling personnel presses a button indicating the shelves in which

a card he wishes to consult is contained.

- The shelves rotate until the correct one arrives at hand level, the operator then look for the appropriate tray for the card. Speedy access to the index here is possible with the operator seated comfortably at the machine compared with the drawers and standing on tip toe to access the top ones.

- Because of the physical conditions, it provides the accuracy of filling and finding cards is enhances. Two factors must be considered here;

  i. Acceptability

  ii. Cost

- The larger machines are expensive to buy and to maintain access is also limited in that only one person can consultant the index at any given time, however this may be turned to an advantage because access to index should only be allowed to those people with legitimate need to do so. This problem can be solved by having 2 or 3 smaller machines instead of one massive one that limits access to one person at a time.

Cards Wheel

- As described by the name, cards are attached to a large upright central wheel which rotated until the required sections come to the top. This type of equipment in however unsuitable for large hospital.

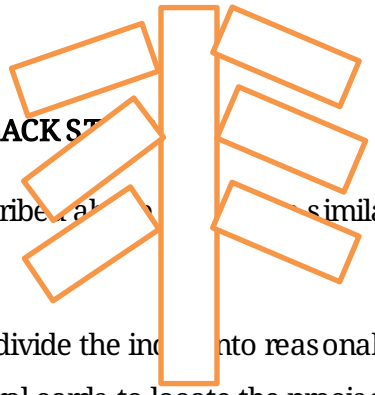
Carousel Type file

- Cards are usually housed in open metal boxes attached to a central upright spindle. These boxes are swung out for use and folded back towards the spindle when finished when finished with.

- The boxes are attached right round the spindle several personnel can gain access to the index at the same time.

- This equipment is claimed to be space saving considering the amount of cards that can be housed.

Examples of carousel type of file



GUIDE CARDS AND BACK ST

-All the systems describe[illegible] similar type of guide cards will the exception of the strip index.

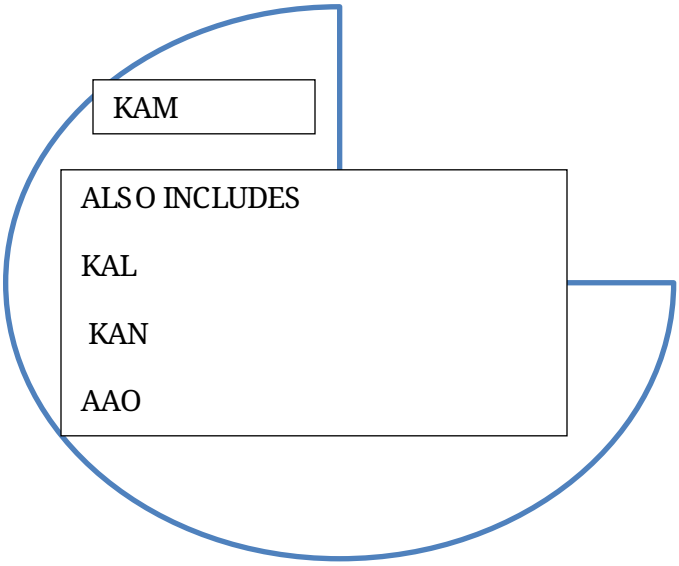
-The guide cards subdivide the in[illegible]nto reasonable sections so that less time is spending limiting through several cards to locate the precise section of the file. A large index will probably need a guide cards for every 200 cards and this should also be considered when calculating the size to which the index will grow.

-Guide cards are usually thicker than the index cards and therefore take up considerable footage in a large index.

- The guide cards should have a tab protruding above the top of the index cards on which can be written a letter or the first three letters of the first card in that section e.g. CLA at the start with the most names, the surname and initial e.g. Kamau A, Kamau D. etc.

- Set of blank guide cards can be handwritten or printed.

-Guide cards may be costly but, with the importance of the master patient index it may be possible to convince the administration to purchase them for you.



BACK STOPS

- These are pieces of metal that stop the cards in drawers which is not full from sliding down
- It is important to determine what type of backstops one is being provided with when buying filling drawers
- The way they function can easily affect the overall operation of the index.
- Backstops should be firm enough to stay in place when a drawer is closed with force, nut mobile enough to be readily adjusted as the cards have to be expanded backwards.

COMPUTERIZED SYSTEMS (MPI)

-Currently there has been more towards computerized systems for the master patient index. Despite the greater interest in this method it has not been employed in almost all health facilities.

-The practice is definitely going to spread in the other hospitals in the future and manual systems as the price of computers is becoming more affordable.

-Access to the computerize systems can take several forms;

1) Direct visual Display Unit (VDU) 1mk with data files
2) Telephone Directory type of index

3) Micro-file record of computer file(COM)

1) Direct visual Display Unit (VDU) 1mk with data files

- Here the computer file will be updated with the addition of each registration and access to the file is very fast and contains less errors i.e. clerical risk.
- The method probably offers the best system in terms of accuracy which would benefit the entire department.

2) Telephone Directory type of index

- This is a volume printed by the computer at regular intervals. Several copies of the index can be produced for use at different registration centres. However some form of secondary or temporary index will have to be maintained in manual form to cover for registrations occurring between updates and this is a major limitation to the primary purpose of the index.

3) Micro-file record of computer file(COM)

- With this method sheets of micro-film contain patients' registrations and are similar to the telephone directory type of index.
- Access to the data is normally via micro-film viewers but again a temporary index has to be manually maintained.
- Shifting from a manual to computerized systems is not easily to all existing cards entered on to the computer can be a very costly exercise and uneconomical too. The various alternatives are;
    - ✓ Converting all entries to the computer file and have a complete index on computer regardless of the cost.
    - ✓ Include only new registration and re-registration on computer file and maintain a manual index of old cases.
    - ✓ Include new registrations and gradually run down the manual index until if eventually becomes non-current.
    - ✓ The problems assisted with the conversation of the index will therefore largely depend on the size of the index and other local circumstance.

FILLING TECHNIQUES USED IN FILLING MASTER PATEINT INDEX CARDS

A. SORTING

- The most important step in filing is sorting which means putting the cards into goods order before being filed
- There are 2 methods of sorting most appropriate to a master index;
    - i. Use of sort file- This is series of flaps each bearing a letter of the alphabet arranged in a solid.
    - ii. Pigeon holes- each lettered for the initial sorting of the index cards.

B. FILING

- This requires a lot of concentration. It easy to file in a card when the files in front and behind have been checked. This should be allocated to an experience clerk.

C. CHECKING

- It is obvious that mistakes do occur when one is filling the cards and therefore constant checking of file cards is required.

Reference to index

- Once cards have been filed and checked they should only be removed when the address needs to be changed.
- Cards should never be so packed as to prevent checking and filing. The most important thing to note here is that the master index cards should be filed strictly alphabetically and only one number of staff should work on the cards at any one time to prevent cards from being misfiled.

FILING METHOD FOR MASTER PATIENT INDEX

- The first item to be considered when the index is to be consulted/ filed is the patient's surname. It would be necessary to decide how surname should be treated because this card must always be filed according to one principle.
- The best way is to regard all parts of a hyphenated or double barren surname as though they were on continuous word e.g.
    - WG-Kamau
    - K' Odhiambo

- Person-Robb e.t.c

- Surname with foreign prefix e.g. de la should be treated as one word e.g.
    - De Haunty
    - De La mare
    - Delamare
- MC. Mae and M should all be treated as mae, the next letter determine the order of entry e.g.
    - MC Donald Alice
    - Mac Donald Authouly
    - MC Danald Anthony
    - Mac Donald Authur
    - MC Elvoy
- Surnames beginning with abbreviation "ST" should be filed after "Saint" e.g.
    - Saintsbury
    - Saint
    - St. Aubyn
    - St. John
    - Sainty
- It is P.M (necessary) that whatever convention you have decided on should be written down and clearly displayed to all who file or consult the index
- When this has been agreed on there are various ways in which he cards may be filed.

a) Strict alphabetical (Surname first and data of birth)

b) Strict alphabetical (Surname and all forenames)

c) Phonetic Treatment of surnames

d) Soundex

e) Separate filing of male and females

a) Strict alphabetical (Surname first and data of birth)

- Where the surname and first fore name are identical all forenames are ignored and filed according to the year of birth the youngest towards the back of the file
    - Odongo Mariom Teresiah 16.01.14
    - Odongo Mariom Mary 24.10.50
    - Odongo Mariom Everlyne 13.09.65
- This is a system that deals effectively with filing of cards containing the most common surnames and first forenames.
- It is easy to identify the correct patient in a file containing several hundred patients with the same names so long as the accurate data of birth is known.

b) Strict alphabetical (Surname and all forenames)

- Where the surname and first names are the same, cards will be filled according to the initial of the second forename;
    - Odongo Mary
    - Odongo Mary Ann
    - Odongo Mary Joan
- Except for the very common names identification can be possible provided all forenames are known when the index is consulted.
- This method is not as accurate as (a) above

c) Phonetic Treatment of surnames

- Where the surnames sound the same cards may be filed in the order of the first name and date of birth e.g.
    - Clark Mary Jane 16.01.14

- Clerk Mary 24.10.50
- Clarke Mary Ann 13.09.65

- This method provides a built in safeguard against careless registration of recording of facts or against a patient who may sometimes vary the spellings of the surname.
- Filing in date of birth order would be the best for this method to succeed.

d) Soundex

- This system was derived by the Americans to deal with patients whose names represent a cross section of almost every nation of the world and pronunciation is more important than spelling in relation to the index. Although the method may not be relevant in our case. It's important that you are aware of the principles involved.
- This system indexes surnames to groups of similar names means of simple numerical control (Code)
- Soundex coding reduces all surnames to a 3 digit number and then the year of birth order is used to file the cards.
- This is the most difficult system to understand and requires more staff training than other methods; e.g.
    - Young Mary Ann 240 16.01.70
    - Young Mary J ane 240 26.17.06
    - Young Mary Lyune 240 01.02.89

e) Separate filing of male and females

- The larger the file grows the more the chance of miss filling and the longer it takes to access any one card.
- This results in the introduction of some modification e.g. the use of different coloured cards for male and female patients' cards such a modification removes the requirement of indicating "Sex" on the cards and enhances faster access to the index.
- This is because when checking for a male patient whose cards is miss filed, the searching personnel will go straight to the drawer containing male cards and not

searching the entire index and would be the case when all cards were of same colour

ADVATANGES AND DISADVANTAGE OF INDEX CARDS FILLING METHODS

SYSTEM	ADVANTAGES	DISADVANTAGES
Alphabetical	Probably the most readily/ quickly/ faster understood system	-Require separate names with sounds the same (Phonetic names e.g. Browne-Brown -Depends on correct spelling of names being used otherwise duplicate registration may occur -It's labour intensive i.e. checking alternative spelling of names
Phonetic	-Brings together names that sound a live e.g. Urwin, Irwin. -Saves time spent on checking alternative spelling of names -Helps prevent duplicate registration	-Needs strict and clear rules to determine where will it be filled in the index e.g. Urwin &Irwin could be filled under "U" or "I" -Staff may be over enthusiastic and extend system to match names not strictly phonetic e.g. Smith and Smythe. -Requires careful supervision
Soundex	-Conversion of names to code brings together similar names and helps prevent duplication of registration	-Most difficult system to understand -Requires more staff training than other methods

APPOINTMENT SYSTEMS

Learning Objectives

The student should be able to;-

i. Define scheduling and follow up

ii. Describe concept of scheduling and follow up

iii. Explain the purpose of scheduling and follow up

iv. Describe the types of appointment systems

v. Describe different sources of appointments

vi. Describe the equipment used to maintain appointment

vii. Explain how to prepare a clinic

Appointment system is a system is a systematic way of giving a specific date, time and venue of the clinic to an individual patient/client.

CONCEPTS OF SCHEDULING

1. Patients should be distributed evenly to various clinics depending on the number that the consultants will be able to see.
2. The staff meaning the clinics should ensure that they are in time so that the patients are not kept waiting for long before they are seen
3. Overloading of clinics should be avoided
4. There should be a laid down policy on how to schedule current cases
5. Staff working in the appointment area should be familiar with the layout of the hospital so that they can give proper direction to the patients/clients.

PURPOSE OF SCHEDULING AND FOLLOW UP

a) To reduce patients' waiting time

b) To provide an even spread of work over the whole clinic session. An appointment provides for even spread of work among the medical staff running the clinic.

c) To allow the hospital to prepare each and every patient in advance so that delay for registration at the time of the clinic is or can be reduced.

d) To provide special clinic arrangements, for example recording of social history, weighing of patients, removal of plaster, pathological and radiological examinations.

e) To allow for distance and known transport difficulties. Allowance should be made by health record staff for such factors as the distance a patient has to travel, train and arrival and departure times, rural transport difficulties, availability of local ambulance to the health care facility. All these can consistently ease burden of attending the hospital.

f) To provide for teaching arrangements. A consultant may wish to select an interesting case to present to the medical students. This is simplified if an appointment system is well organized in the health institution.

TYPES OF APPOINTMENT SYSTEMS

There are 3 types

A. CENTRALIZED

B. DECENTRALIZED

C. COMBINED

A. CENTRALIZED

This means that all appointments for the various clinics are made in one central place. (Same work being done in one area)

ADVANTAGES

i. The master index card will be near the area for quick reference

ii. Control of staff, stationery and equipment by the health records & Information Officer is easy.

iii. Each of the appointment clerks becomes familiar with the working systems of the various consultants.

iv. All inquiries concerning appointments are referred to one place.

v. Urgent cases are channeled quickly to the respective clinics.

vi. When one clerk is sick or goes on leave, the other clerk continuous with the work since they are all familiar with the work.

B. DECENTRALIZED

This is an appointment system which is carried on different clinics.

ADVANTAGES

i. The clerk dealing with the appointments becomes familiar with the patients and knows them by name.

ii. The consultant of the clinic in charge of the clinic will know the number of patients on his list without having to walk or ring the central area.

DISADVANTAGES

i. The master index is far from the appointment area.

ii. Control of available resources is difficult

iii. When the clerk who mans the clinic becomes sick or is on leave, the work come to a stand still

iv. Enquiries concerning appointments are directed to different places

C. COMBINED

Some of the hospital has the two systems combined. Return appointments being made in the various clinics and new bookings being done in the central area. Therefore it is the duty of health records/information officer to look at these systems critically to decide on which suits his/her hospital or health institution.

SOURCES OF REQUEST FOR APPOINTMENTS

a) Patients telephoning personally to make appointments

b) Physicians on private practice wanting to book appointments for their patients through telephone or writing.

c) Letters from health institution given to patients to come and book their own appointments

d) Patients already discharged from the hospital making return appointment

e) Patients referred from other clinics or from one clinic to another.

EQUIPMENTS USED TO RECORDS APPOINTMENTS

-The choice of the equipment used will depend on whether the appointment is being made centrally or de-centrally.

a) Loose leaf binder

b) Visible edge sheets

c) Diaries

a) Loose leaf binder

- Books vary in size and it is recommended that they be in loose leaf form to facilitate additions and removal of the sheets. The type used depends on local circumstances.

b) Visible edge sheets

- Sheets are cut out and by the use of cardex, these sheets are fixed on the cardex and the information written on them can easily be seen when the cardex is open.

c) Diaries

- Books that are bound can be used to write all the appointments for one consultant. The disadvantage is that one keeps cancelling the list making the diary to become untidy. Another disadvantage is that one cannot photocopy the diary direct. He has to write out other list from the diary.

CLINIC PREPARATION

- Two days prior to the clinic, all the clinic list should be sent to the appropriate sections;-
    - One to the clinic
    - One to the filling area
    - Another to the consultant in charge of the clinical area.
- All the records and documents should be pulled out, all the pathological reports inserted in the correct files and kept in the pigeon holes for the clinic receptionist to come and collect.
- The clinic receptionist should make sure that all the documents such as x-rays are ready for each and every patient attending the clinic.
- The clinic are prepared two days in advance to reduce the patients waiting time before

he/she is seen by the physician

- At the end of the clinic the receptionist should ensure that all the files are returned to the central library ready for filling.